

* Final Report *

ICD-6-3
Carcinoma, urothelial papillary
Site ^{CHF} Bladder NOS 813013
^{path} C67.9
Bladder, posterior wall C67.4
JW 3/26/14

Document Type:

*Date - Date of Service:

Document Status:

Document Title:

Encounter info:

Contributor system:

UUID:82C3CDC8-DC42-4971-9203-308712F89553

TCGA-XF-A9SU-01A-PR

Redacted



* Final Report *

ORDERING PHYSICIAN:

Ordering Physician:

PRE-OPERATIVE DIAGNOSIS:

Bladder CA

POST-OPERATIVE DIAGNOSIS:

Same

MICROSCOPIC DESCRIPTION:

A-D: See final microscopic-diagnosis.

E: Sections of the bladder show multiple extensive areas of high-grade invasive urothelial carcinoma, penetrating into perivesical fat but sparing adjacent organs (uterus and cervix). Extensive multifocal lymphovascular space invasion is present. Also present are several foci of the deceptively bland trabecular pattern superficially invading the lamina propria.

Sections of the cervix show cervical intraepithelial neoplasia (CIN III) with gland duct involvement. No invasive cervical squamous cell carcinoma is identified. Architecturally while the urethra is involved with urothelial carcinoma, the ectocervix and cervical stroma are not, thus the two processes are physically discontinuous.

Signature Line

Signed by:

ELECTRONIC SIGNATURE

GROSS DESCRIPTION:

A: The specimen is received fresh from the O.R. for rapid frozen analysis and labeled "Rt. distal ureter". It consists of a segment of ureter measuring 0.6 x 0.5 x 0.2 cm. The specimen is submitted entirely for frozen section.

B: The specimen is received fresh from the O.R. for rapid frozen analysis and labeled "Lt. distal ureter". It consists of a segment of ureter measuring 0.6 x

Printed by:

Printed on:

Page 1 of 8
(Continued)

* Final Report *

0.6 x 0.5 cm. One cassette is submitted for frozen section and one cassette is submitted for permanent section.

C: The specimen is received fresh from the O.R. for rapid frozen analysis and labeled "Inter aortal caval upper limits of dissection". It consists of lymph node measuring 2 x 2 x 0.5 cm. One cassette is submitted for frozen section and one cassette is submitted for permanent section.

D: The specimen is received fresh from the O.R. for rapid frozen analysis and labeled "Apical urethral margin F/S". It consists of section of mucosa measuring 4 x 0.3 x 0.2 cm. The specimen is submitted entirely for frozen section.

E: The specimen is received fresh from the O.R. for intraoperative consultation and labeled "Bladder". It consists of radical cystectomy specimen measuring 11 x 13 x 4.5 cm. The peritoneal surface measures 13 x 8 x 0.1 cm. There is no puckering or erythema. The bladder measures 7 x 7 x 2 cm. The bladder is opened along its anterior aspect to reveal several ulcerations with overlying pseudo membranes as well as a pedunculated lesion. The largest ulcer measures 4 x 3 cm and extends grossly into to perivesical fat. A 2.5 x 2 cm ulcer is located superior to the right ureteral orifice. A 1.5 x 1 cm ulcer is located in the right aspect of the dome. A 1.5 x 1 cm ulcer is located inferior to the left ureteral orifice and 1 x 0.5 cm ulcer is located at the urethral margin. The pedunculated lesion measures 2 x 1 cm and is located in the left anterior aspect of the dome of the bladder. The mucosa, which is uninvolved with ulceration, is inflamed and edematous. The right distal ureter measures 5.5 cm in length and 0.5 cm in circumference. The left distal ureter measures 4 cm in length and 1 cm in circumference and contains a stent. Received in continuity with the specimen is 12 x 8 x 2 cm uterus and adnexa. The uterus measures 5 x 8 x 2 cm and is pear shaped. The uterine serosa is smooth. The right fallopian tube measures 5.5 cm in length and 0.5 cm in diameter. The right ovary measures 2 x 1 x 0.5 cm and is grossly unremarkable. The left fallopian tube measures 7.5 cm in length and 0.5 cm in diameter. The left ovary measures 1.5 x 1 x 0.5 cm and is grossly unremarkable. Several small peritubal cysts, which are clear and thin-walled, are noted on the surface of the left fallopian tube. The ectocervix measures 3.5 cm in greatest diameter. The cervical os measures 0.8 cm and is unremarkable. The uterus is opened to reveal a 3.5 cm endocervical canal lined with mucoid tan epithelium and a 4.5 x 1.5 x 0.1 cm endometrial canal lined by a velvety tan endometrium. The myometrium displays the usual pink-tan trabeculated appearance interrupted by a single myoma located intramurally and measuring 1.2 cm in greatest diameter. There is no hemorrhage or necrosis of the myoma. Representative sections submitted entirely in twenty-five cassettes.

F: The specimen is received in formalin and labeled "Right para caval lymph nodes". It consists of a portion of node bearing adipose tissue measuring 3.5 x 2.3 x 2 cm. The specimen is submitted entirely in one cassette.

* Final Report *

G: The specimen is received in formalin and labeled "Right common iliac nodes". It consists of a portion of node bearing adipose tissue measuring 7 x 3.6 x 1.2 cm. The specimen is submitted entirely in two cassettes.

H: The specimen is received in formalin and labeled "Left para aortic lymph nodes". It consists of a portion of node bearing adipose tissue measuring 2.5 x 2 x 1 cm. The specimen is submitted entirely in one cassette.

I: The specimen is received in formalin and labeled "Left common iliac lymph nodes". It consists of a portion of node bearing adipose tissue measuring 4.3 x 2.3 x 1 cm. The specimen is submitted entirely in one cassette.

J: The specimen is received in formalin and labeled "Right lymph node of Cloquet". It consists of a portion of node bearing adipose tissue measuring 2.3 x 1.2 x 1 cm. The specimen is submitted entirely in one cassette.

K: The specimen is received in formalin and labeled "Right external iliac lymph nodes". It consists of a portion of node bearing adipose tissue measuring 4.5 x 4.2 x 1.5 cm. The specimen is submitted entirely in two cassettes.

L: The specimen is received in formalin and labeled "Right obturator/hypogastric lymph nodes". It consists of a portion of node bearing adipose tissue measuring 5.5 x 4 x 2 cm. The specimen is submitted entirely in three cassettes.

M: The specimen is received in formalin and labeled "Left lymph node of Cloquet". It consists of a portion of node bearing adipose tissue measuring 2.2 x 1.5 x 1 cm. The specimen is submitted entirely in one cassette.

N: The specimen is received in formalin and labeled "Left external iliac lymph nodes". It consists of a portion of node bearing adipose tissue measuring 5 x 3 x 1 cm. The specimen is submitted entirely in one cassette.

O: The specimen is received in formalin and labeled "Left obturator/hypogastric lymph nodes". It consists of a portion of node bearing adipose tissue measuring 5 x 5 x 0.5 cm. The specimen is submitted entirely in three cassettes.

P: The specimen is received in formalin and labeled "Right presciatic lymph nodes". It consists of a portion of node bearing adipose tissue measuring 1 x 1 x 0.5 cm. The specimen is submitted entirely in one cassette.

Q: The specimen is received in formalin and labeled "Presacral lymph nodes". It consists of a portion of node bearing adipose tissue measuring 2 x 1.5 x 1 cm. The specimen is submitted entirely in one cassette.

* Final Report *

R: The specimen is received in formalin and labeled "Left presciatic lymph nodes". It consists of a portion of node bearing adipose tissue measuring 1 x 1 x 0.5 cm. The specimen is submitted entirely in one cassette.

S: The specimen is received in formalin and labeled "Urethra". It consists of a cylindrical tan tissue fragment lined on the inside by mucosa and on the outside by fibrous tissue measuring 3.5 x 2 x 2 cm. A stitch is present indicating the proximal aspect of the specimen. The specimen is radially inked black. Serial sections are submitted entirely in five cassettes.

T: The specimen is received in formalin and labeled "Left proximal ureter". It consists of section of ureter measuring 2 x 1 x 0.3 cm. The specimen is submitted entirely in one cassette.

U: The specimen is received in formalin and labeled "Right proximal ureter". It consists of section of ureter measuring 2 x 1 x 0.3 cm. The specimen is submitted entirely in one cassette.

SECTIONS:

AFS: frozen section, rt. distal ureter
ART: remaining tissue embedded
BFS: frozen section, lt. distal ureter
BRT: remaining tissue embedded
CFS: inter aortal caval upper limits of dissection embedded
CRT: remaining tissue embedded
DFS: frozen section, apical urethral margin F/S
E1: right ureter specimen margin
E2: left ureter specimen margin
E3: right ureterovesical junction
E4: left ureterovesical junction
E5: trigone, bladder neck
E6: ulcer at urethral specimen border
E7-8: right posterior large ulcer
E9-10: right posterior large ulcer, junction with adjacent urothelium
E11: right posterior smaller ulcer
E12: pedunculated lesions tip
E13-14: pedunculated lesion, remaining
E15: dome
E16: anterior cervix
E17: posterior cervix
E18: anterior endo myometrium to serosa with myoma
E19: posterior endo myometrium to serosa with myoma
E20: right adnexa
E21: left adnexa
E22-23: right perivesical fat

Printed by:
Printed on:

Page 4 of 8
(Continued)

* Final Report *

E24-25: left perivesical fat
F: right para caval lymph nodes embedded
G1-2: right common iliac nodes embedded
H: left para aortic lymph nodes embedded
I: left common iliac lymph nodes embedded
J: right lymph node of Cloquet embedded
K1-2: right external iliac lymph nodes embedded
L1-3: right obturator/hypogastric lymph nodes embedded
M: left lymph node of Cloquet embedded
N: left external iliac lymph nodes embedded
O1-3: left obturator/hypogastric lymph nodes embedded
P: right presciatic lymph nodes embedded
Q: presacral lymph nodes embedded
R: left presciatic lymph nodes embedded
S1: margin
S2-5: serial sections from distal to proximal
T: left proximal ureter embedded
U: right proximal ureter"

INTRAOPERATIVE DIAGNOSIS:

GROSS DIAGNOSIS:

Bladder (E):

tumor present near urethral margin

DIAGNOSIS COMMENT:

Carcinoma in situ with pagetoid spread is noted in the cervix (part E) and ureterectomy specimen (part S). We will perform further studies in an attempt to determine if the two processes are related or, are separate, and these results will be reported in an addendum.

SEE Addendum - related and of transitional cell type.

FINAL DIAGNOSIS:

RADICAL CYSTECTOMY AND HYSTERECTOMY WITH BILATERAL SALPINGO-OOPHORECTOMY:

RIGHT DISTAL URETER (A):

FROZEN DIAGNOSIS:

- NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER

- NO DYSPLASIA OR MALIGNANCY IDENTIFIED

Printed by:

Printed on:

Page 5 of 8
(Continued)

* Final Report *

LEFT DISTAL URETER (B):

FROZEN DIAGNOSIS:

- NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER SHOWING REACTIVE ATYPIA
- NO DYSPLASIA OR MALIGNANCY IDENTIFIED

INTER AORTAL CAVAL UPPER LIMITS OF DISSECTION (C):

FROZEN DIAGNOSIS:

- NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- NO EVIDENCE OF MALIGNANCY IN SIX LYMPH NODES (0/6)

APICAL URETHRAL MARGIN (D):

FROZEN DIAGNOSIS:

- CARCINOMA IDENTIFIED

FINAL DIAGNOSIS:

- INVASIVE UROTHELIAL CARCINOMA EXTENSIVELY INVOLVING SUBMITTED TISSUE

BLADDER, UTERUS, BILATERAL FALLOPIAN TUBES AND OVARIES WITH RIGHT AND LEFT PERIVESICAL LYMPH NODES (D):

BLADDER (D)

GROSS DIAGNOSIS:

- TUMOR ULCER PRESENT AT URETHRAL MARGIN

FINAL DIAGNOSIS::

- INFILTRATING UROTHELIAL CARCINOMA, HIGH GRADE WITH FOCAL TRABECULAR AND NESTED FEATURES, NUCLEAR GRADE 3-4/4 WITH INVASION THROUGH MUSCULARIS PROPRIA AND GROSSLY INTO PERIVESICAL FAT
- MULTIFOCAL CARCINOMA IN SITU
- HIGH GRADE PAPILLARY UROTHELIAL CARCINOMA
- EXTENSIVE, MULTIFOCAL LYMPHOVASCULAR SPACE INVASION IDENTIFIED
- TUMOR PREDOMINANTLY INVOLVES THE LEFT POSTERIOR BLADDER WALL
- URETHRAL MARGIN IS POSITIVE FOR CARCINOMA (SEE PART D)
- THE FINAL URETHRAL MARGIN IS INVOLVED BY CARCINOMA IN SITU (SEE PART S)
- INVASIVE TUMOR DOES NOT INVOLVE CERVIX OR UTERUS, HOWEVER, CERVIX SHOWS CARCINOMA IN SITU (SEE COMMENT)
- BILATERAL URETER MARGINS ARE NEGATIVE WITH LEFT URETER SHOWING SCAR AND REACTIVE ATYPIA SECONDARY TO STENT PLACEMENT

UTERUS, BILATERAL FALLOPIAN TUBES AND OVARIES:

- CERVIX SHOWING CARCINOMA IN SITU WITH PAGETOID SPREAD
- ENDOMETRIUM, ATROPHIC ENDOMETRIUM AND MYOMETRIUM
- SMALL LEIOMYOMATA

Printed by:
Printed on:

Page 6 of 8
(Continued)

* Final Report *

- SEROSA SHOWING NO SIGNIFICANT PATHOLOGIC CHANGE
- BILATERAL OVARIES AND FALLOPIAN TUBES SHOWING PHYSIOLOGIC ATROPHY

RIGHT AND LEFT PERIVESICAL LYMPH NODES:

- NO LYMPH NODES OR TUMOR IDENTIFIED (0/0)

RIGHT PARACAVAL LYMPH NODES (F):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN SIX LYMPH NODES (0/6)

RIGHT COMMON ILIAC LYMPH NODES (G):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN EIGHT LYMPH NODES (0/8)

LEFT PARAAORTIC LYMPH NODES (H):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES (0/7)

LEFT COMMON ILIAC LYMPH NODES (I):

- METASTATIC CARCINOMA IDENTIFIED IN ONE OF FOUR LYMPH NODES (1/4)

RIGHT LYMPH NODE OF CLOQUET (J):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN ONE LYMPH NODE (0/1)

RIGHT EXTERNAL ILIAC LYMPH NODES (K):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN FIVE LYMPH NODES (0/5)

RIGHT OBTURATOR/HYPOGASTRIC LYMPH NODE (L):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES (0/7)

LEFT LYMPH NODE OF CLOQUET (M):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN FOUR LYMPH NODES (0/4)

LEFT EXTERNAL ILIAC LYMPH NODES (N):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN ONE OF NINE LYMPH NODES (1/9)

LEFT OBTURATOR/HYPOGASTRIC LYMPH NODES (O):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN 12 LYMPH NODES (0/12)

RIGHT PRESACRAL LYMPH NODES (P):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN TWO LYMPH NODES (0/2)

PRESACRAL LYMPH NODES (Q):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN 13 LYMPH NODES (0/13)

LEFT PRESACRAL LYMPH NODES (R):

- NO EVIDENCE OF MALIGNANCY IDENTIFIED IN TWO LYMPH NODES (0/3)

Printed by:
Printed on:

Page 7 of 8
(Continued)

* Final Report *

URETHRA (S):

- EXTENSIVE CARCINOMA IN SITU IDENTIFIED WITH PAGETOID SPREAD
- CARCINOMA IN SITU INVOLVES DISTAL RESECTION MARGIN
- RADIAL INKED SURGICAL MARGINS, ARE NEGATIVE (SEE COMMENT)

LEFT PROXIMAL URETER (T):

- BENIGN URETER
- NO IN SITU OR INVASIVE CARCINOMA IDENTIFIED

RIGHT PROXIMAL URETER (U):

- BENIGN URETER
- NO IN SITU OR INVASIVE CARCINOMA IDENTIFIED

TNM STAGE: pT3bN1MX

LYMPH NODE SUMMARY: Tumor identified in 2 of 87 lymph nodes (2/87)

Printed by:
Printed on:

Page 8 of 8
(End of Report)

HISTORY ADDENDUM REPORT

* Final Report *

Document Type:

*Date - Date of Service:

Document Status:

Document Title:

Encounter info:

Contributor System:

* Final Report *

DIAGNOSIS:

Carcinoma-in-situ with pagetoid spread was noted in the patient's bladder, urethra, and also in the cervical specimen. In order to determine the origin of these processes, and whether or not they were related or separate, we performed immunohistochemical studies on the bladder tumor (slide E7), the cervix (slide E17), and the urethra (slide S2). The studies demonstrate the following:

	E7 Bladder	E17 Cervix	S2 Urethra
CK7	++	++	++
		+/- squamous mucosa	
CK20	+	+	+
		- squamous mucosa	
CPV	-	-	-
	High background	High background	High background

These results indicate that the carcinoma in situ process is related, and of transitional cell origin. Thus, the transitional cell carcinoma is extending by pagetoid spread widely throughout the anterior exenteration specimen.

88342 x 9

Signature Line

Signed by:

ELECTRONIC SIGNATURE

Completed Action List:

Printed by:

Printed on:

Page 1 of 1
(End of Report)

Criteria	lw 3/3/14	Yes	No
Diagnosis Discrepancy			
Primary Tumor Site Discrepancy			
HIPAA Discrepancy			
Prior Malignancy History			
Dual/Synchronous Primary Noted			
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initials		Date Reviewed: 3/3/14	